

CLINICAL INTELLIGENCE TREND REPORT

GREEN: Routine Review

Patient ID: NancyPeriod: April 25 to July 24, 2024Report Date: July 25, 2024Coverage: 1315/2184h (60%)

Decision-support summary derived from longitudinal vital sign trends; interpret in clinical context.

Clinical Alerting Thresholds

Very Low HR < 40 bpm

Very High HR > 110 bpm

Low HR < 45 bpm

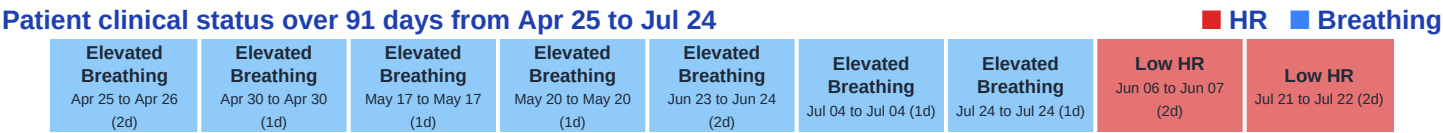
Elevated Breathing > 24 brpm

Elevated HR > 95 bpm

High Breathing > 30 brpm

High HR > 100 bpm

Very High Breathing > 40 brpm



Last 24 Hours

Last 24 Hours: NORMAL

30-Day Tier: GREEN

| Vital      | Avg     | Min     | Max     |
|------------|---------|---------|---------|
| Heart Rate | 69 bpm  | 49 bpm  | 90 bpm  |
| Breathing  | 17 brpm | 10 brpm | 39 brpm |

Limited data in the final 24h (14/24h); values reflect available readings.

Last 24 hours: one elevated breathing episode overnight, otherwise within normal range.

Episodic Events (last 24h)

| Date   | Time Span    | Duration | Episodes | Condition          | Avg | Min | Max | Comment                                                                  |
|--------|--------------|----------|----------|--------------------|-----|-----|-----|--------------------------------------------------------------------------|
| Jul 24 | 1 AM to 2 AM | 1h       | 1        | Elevated Breathing | 24  | 18  | 39  | Check SpO2 and lung sounds; assess for fluid overload or early infection |

19 episodic events Detected, spanning 19h total. Conditions: Elevated Breathing, Low Heart Rate. Priority grade: Low

| Episodic Events | Total Hours | Highest Burden | Episodes/day | Coverage |
|-----------------|-------------|----------------|--------------|----------|
| 19              | 19h         | Low Heart Rate | <1           | 60%      |

Major Findings: Stable baseline

| Date   | Time Span     | Duration | Episodes | Condition          | Avg | Min | Max | Comment                                                                         |
|--------|---------------|----------|----------|--------------------|-----|-----|-----|---------------------------------------------------------------------------------|
| Jun 06 | 9 PM to 10 PM | 2h       | 2        | Low Heart Rate     | 45  | 42  | 48  | Check pulse and BP; review beta blockers and AV node agents; ECG if symptomatic |
| Jul 21 | 3 AM to 4 AM  | 7h       | 7        | Low Heart Rate     | 44  | 40  | 49  | Check pulse and BP; review beta blockers and AV node agents; ECG if symptomatic |
| Apr 25 | 9 PM to 10 PM | 3h       | 3        | Elevated Breathing | 25  | 23  | 26  | Check SpO2 and lung sounds; assess for fluid overload or early infection        |
| Apr 30 | 9 PM to 10 PM | 1h       | 1        | Elevated Breathing | 25  | 22  | 27  | Check SpO2 and lung sounds; assess for fluid overload or early infection        |
| May 17 | 4 PM to 5 PM  | 1h       | 1        | Elevated Breathing | 27  | 27  | 27  | Check SpO2 and lung sounds; assess for fluid overload or early infection        |
| May 20 | 12 PM to 1 PM | 1h       | 1        | Elevated Breathing | 24  | 24  | 25  | Check SpO2 and lung sounds; assess for fluid overload or early infection        |

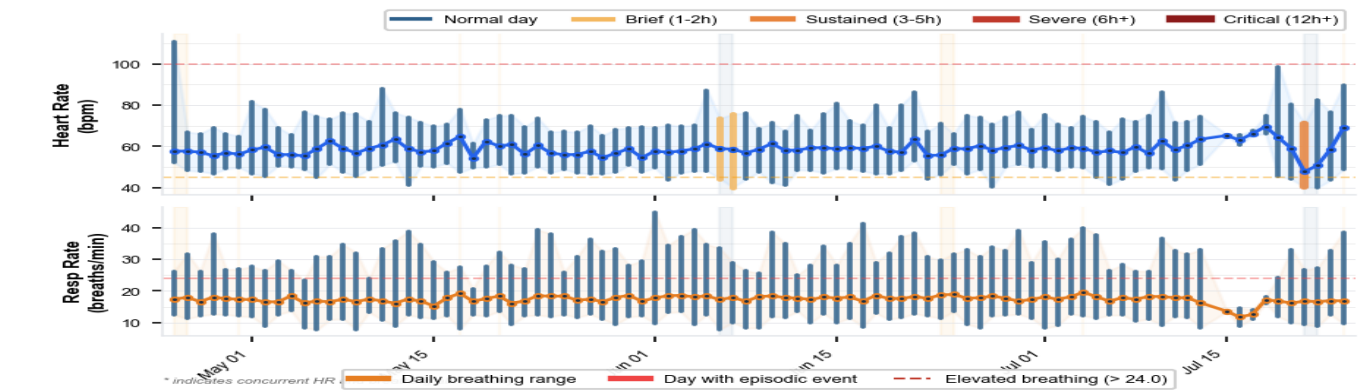
+ 3 additional condition(s) across Elevated Breathing; details in monitoring data.

Suggested Clinical Review Actions

- Low Heart Rate (avg 44 bpm, min 40 bpm): Check pulse and BP; review beta blockers and AV node agents; ECG if symptomatic
- Elevated Breathing (avg 25 brpm, peak 26 brpm): Check SpO2 and lung sounds; assess for fluid overload or early infection
- Overall trajectory shows 9 unstable phases with 18 bpm variability. Consider increasing monitoring frequency and lowering escalation threshold.

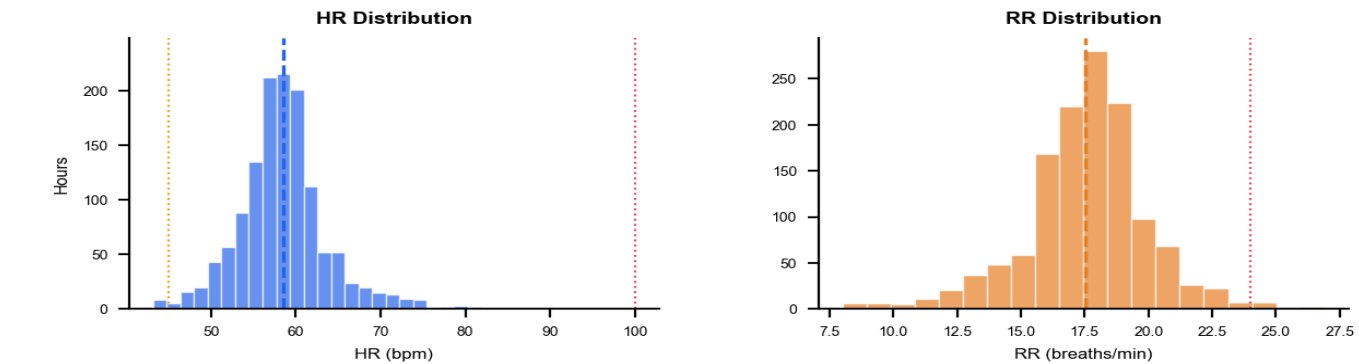
Clinical Pattern Observations:

- 1. **Clustered pattern:** Episodic events on 3 of 91 days (3%).
- 2. **Nocturnal pattern:** 4 of 6 eligible episodes during evening or night hours.

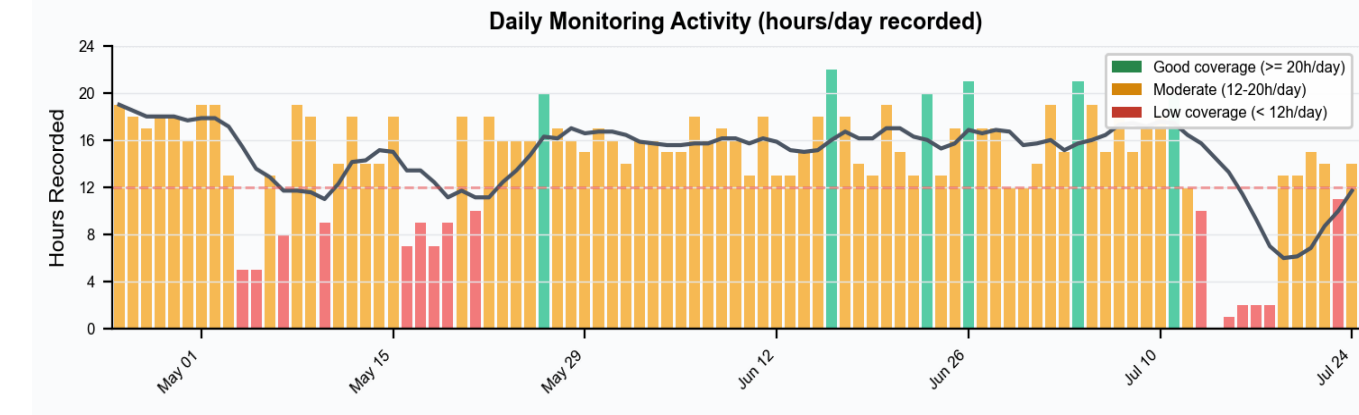


Red bars indicate days with detected episodic events.

Vital Sign Distribution



Monitoring Activity



Bars indicate monitoring hours/day. Red threshold line indicates clinical monitoring baseline.

■ HR episode ■ RR episode ■ Recorded, no episode ■ No data

Episodic event: threshold exceeded continuously for  $\geq 1$  hour. Episodes separated by  $\leq 1$  hour of normal vitals are counted as the same event. Brief threshold crossings that do not sustain are not flagged.